

For the use only of a Registered Medical Practitioner or a Hospital or a Laboratory.

LASTUSS (LA)

COUGH SUSPENSION

(Dextromethorphan Hydrobromide Oral Suspension)

COMPOSITION

Each 5 ml contains
Dextromethorphan in a polymer matrix base,
Equivalent to Dextromethorphan
Hydrobromide B.P.30 mg
Preservative:
Methylparaben BP9 mg
Propylparaben BP1 mg
Permitted colour: Sunset Yellow FCF

DESCRIPTION:

Orange coloured suspension, pleasantly flavored, having sweet taste.

PHARMACOLOGICAL ACTION

Pharmacodynamics:

Dextromethorphan hydrobromide has been demonstrated to be an effective Antitussive doses between 15 and 60 mg twice daily in a number of doubleblind clinical trial.

Dextromethorphan is well absorbed after oral administration. Plasma levels of unmetabolized dextromethorphan following therapeutic doses are very low due to extensive biotransformation and tissue distribution which remove the drug rapidly from the blood. The plasma level of Dextrophan the major metabolite of Dextromethorphan is measured using fluorescence spectrophotometry after administering 30 mg. Dextromethorphan orally in syrup. Dextrophan orally in syrup. Thus, peak plasma Dextrophan levels were reached two hours after administration of Dextromethorphan.

In man, following ingestion of 50 - 180 mg. Dextromethorphan, the urinary excretion including metabolites accounted for 25 to 56% of the ingested dose, faecal excretion for 0.12%. Dextrophan is the main metabolite of Dextromethorphan. The presence of morphine and codeine in the urine was definitely excluded. About 8% of a dose is excreted as unchanged drug in 6 hours.

On account of the difficulties in measuring the low plasma levels of Dextromethorphan in humans, the plasma half-life has been estimated in dogs. Following i.v. administration of 2mg./kg. The plasma levels of unmetabolized Dextromethorphan declined biexponentially, the terminal exponential phase having half-life of 2.5 - 4 hours.

INDICATIONS:

For dry and unproductive cough.

RECOMMENDED DOSAGE, DOSAGE SCHEDULE AND ROUTE OF ADMINISTRATION:

Adults 2 teaspoonsful twice a day.
Children : (6- 12 years)
1 teaspoonful twice a day.
Children : (2-6 years)
Half teaspoonful twice a day or as directed by the Physician.

CONTRAINDICATIONS:

Hypersensitivity to Dextromethorphan. Persistent or chronic cough accompanied by excessive, secretions. In patients receiving monoamine-oxidase inhibitors. Hepatic function impairment.

PRECAUTIONS/WARNINGS:

To be sold by Retail on the prescription of a Registered Medical Practitioner only. Shake thoroughly before use.

Not suitable for use in children below 2 years old.

To be used cautiously and to follow the advice of physician or pharmacist for children between the ages of 2 to 6 years

Interactions with other medicaments:

Alcohol and drugs causing CNS depression should be avoided when taking dextromethorphan.

Pregnancy and Lactation:

There are no specific data on their use during pregnancy. Caution should therefore be exercised by balancing the potential benefit of treatment to the mother against possible hazards to the developing foetus. Excreted in breast milk.

SIDE EFFECTS/ADVERSE REACTIONS:

Occasional nausea, drowsiness, dizziness

Symptoms and Treatment of Overdose:

Dextromethorphan has a low order of toxicity with potential for toxic effects Following acute overdosage being extremely low. Although a few cases of toxicity have been reported, doses in excess of 100 times the usual adult dose have not been fatal.

Manifestations following acute over-dosage of dextromethorphan have included nausea, Vomiting, drowsiness, dizziness, blurred vision, nystagmus, ataxia, shallow respiration, urinary retention, stupor, toxic psychosis and coma.

In a 13 week chronic toxicity programme, 100 mg of dextromethorphan had no influence on the haematological picture and no important gross and microscopical findings in Sprague-Dawley rats. Fourteen week chronic toxicity tests (0.25, 1.0 and 4.0 mg/kg body weight, orally five days per week) in dogs had no side effect on general behaviour, appetite, weight, respiratory and pulse rates, Red and white cell counts or haemoglobin Values. Here is significant gross or microscopic pathological changes at all three dose levels

STORAGE CONDITIONS, USER INSTRUCTION AND PHARMACEUTICAL PRECAUTION:

Keep in a cool, dark place below 30° C

PACKING/PACK SIZES:

Amber coloured bottles of 100 ml packed into a carton

SHELF-LIFE: 24 months

Manufactured in India by:

(FDC) FDC Limited

Regd. Office: B-8, MIDC Industrial Area,
Wajuj, Aurangabad - 431 136,
Maharashtra.

Imported by:

Unimed Sdn. Bhd
53, Jalan Tembaga SD 5/2B,
Bandar Sri Damansara,
52200, Kuala Lumpur, Malaysia

2000005412

EXPORT	RA	QA	R&D (F)	Packaging Development Department	Checked by	Approved by

HISTORY: 1) Change in text matter.
2) New Registration No. Added.

Mini Pharma Code No.: Front: 1672/Back: 1673		LEAFLET		FDC Limited	
Size	110 mm x 144 mm	Unfolded Size		No. of Folds	
GSM	60 gsm, Maglitho paper	Paper		Specifican No.	
Colour	PANTONE Process Black C	Artwork Code : 2000005412	Supercodes AW Code: W11TL02AESMY	Location : Wajuj - Country : Malaysia	Prepared On: 28-09-2016